

Common Elbow Injuries and Conditions

Lateral Epicondylitis (“Tennis Elbow”)

Injury overview	Common in tennis players and golfers, this condition develops due to chronic inflammation of the common extensor tendon that originates at the lateral epicondyle. Improper use of the equipment and poor movement techniques can contribute to the onset of lateral epicondylitis.
Surgical and rehabilitative management	Surgical repair is generally not warranted, and the condition improves rapidly with activity modulation and gradual rehabilitation programs.
Return-to-play exercise and conditioning considerations	Include exercises that stress wrist extension in conjunction with varying elbow flexion/extension angles, pronation/supination, and full extension of the fingers.
Return-to-play criteria	Full pain-free ROM, symmetrical grip strength. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test).
Estimated timeline	Acute flare-ups generally resolve within 3 days. Chronic conditions may require significant activity/lifestyle modification to allow the injured tissue to heal and remodel effectively. These more complicated cases can take 4–6 months.

Medial Epicondylitis (“Golfer’s Elbow”)

Injury overview	This condition involves the common flexor tendon originating on the medial epicondyle. Though its is popularly termed “golfer’s elbow,” any athlete who participates in activities that produce valgus forces to the medial elbow (e.g., throwing, golf swing) can experience medial epicondylitis.
Surgical and rehabilitative management	Surgical repair is generally not warranted, and the condition improves rapidly with activity modulation and gradual rehabilitation programs.
Return-to-play exercise and conditioning considerations	Avoid (initially) isolated wrist flexion training. Nearly every exercise commonly performed in a sports performance training program involves wrist (and hand) flexion elements. Therefore, additional isolated work in this area is not recommended—including spring-loaded grip trainers and gyroscopic modalities. Supplement existing forearm flexor training with exercises to stress the static crush-grip (i.e., bodyweight hang on pull-up bar) and pinch-grip (i.e., firefighter’s carry holding two 10-pound barbell plates together in the same hand).
Return-to-play criteria	Full pain-free ROM, symmetrical grip strength. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test). Standard throwing mechanics (if applicable).

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Estimated timeline	Acute flare-ups generally resolve within 3 days. Chronic conditions may require significant activity/lifestyle modification to allow the injured tissue to heal and remodel effectively. These more complicated cases can take 4–6 months.
Ulnar Collateral Ligament Sprain	
Injury overview	The ulnar collateral ligament provides stability to the medial aspect of the elbow and is commonly injured in athletes participating in throwing sports (e.g., baseball, softball, volleyball, javelin). When the stabilizing ligament fails, the medial aspect of the joint is expanded, placing additional stress on the muscular, neural, and vascular tissues.
Surgical and rehabilitative management	The surgical repair, commonly termed the “Tommy John procedure,” is named after the first professional baseball player to successfully return to a high level of performance following the surgery. Surgical and rehabilitative outcomes routinely return athletes to their previous level of participation. Valgus stress to the elbow is avoided for the first 4–5 months following surgery.
Return-to-play exercise and conditioning considerations	All athletes recovering from elbow injuries should undertake a comprehensive training program to develop stability and mobility of the shoulder complex, in addition to total integration with the trunk, legs, and back.
Return-to-play criteria	Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test).
Estimated timeline	9–12 months is common among athletes with surgical repair. This will fluctuate considering the valgus overload forces required of their sport/position.
Superior Labrum Anterior–Posterior (SLAP) Lesions	
Injury overview	The labrum is a cartilage tissue that attaches to the glenoid of the humerus. It extends the surface area of the glenoid’s small bony surface and passively increases the dynamic joint stability of the glenohumeral joint. A common tear in this tissue is a SLAP lesion. These lesions originate at the superior glenoid (in addition to the long head of the biceps tendon) and run from an anterior-to-posterior direction. Such injuries destabilize the glenohumeral joint and produce pain during high-velocity throwing, requiring rotation ROMs near extreme limits.
Surgical and rehabilitative management	Surgical repair is recommended to restore structural joint stability. Rehabilitation begins following a 3- to 4-week immobilization phase and progresses from that point to improve flexibility, mobility, and stability in a gradual way.

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Return-to-play exercise and conditioning considerations	Avoid early overhead activities and delay throwing (if applicable) until clearance from the medical team.
Return-to-play criteria	Normal, pain-free ROM, strength, and endurance. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test).
Estimated timeline	4–6 months, depending on the requirement for the injured area to participate in high-velocity or high-compression activities.